SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Paclitaxel Days 1,8 and 15 · relapsed or refractorySCLC

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Lung · relapsed/refractorysmall-cell lung cancer · NCCP 00621 · version 3 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Treatment aims to control small-cell lung cancer after previous therapy and may help symptoms. The evidence is limited, so discuss likely benefit, burdens and alternatives with the team.

Schedule
Paclitaxel is given on Days 1,8 and 15 of each 28-day cycle. Day 22 is a planned week off, not a fourth infusion. Treatment continues while helping and tolerated; there is no fixed cycle limit.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Paclitaxel

TAXANE CHEMOTHERAPY

- ☐ Low blood counts / infection
- ☐ Sensory or motor neuropathy, potentially persistent
- ☐ Hypersensitivity / infusion reaction
- ☐ Hair loss and nail / skin changes
- ☐ Muscle and joint aches
- ☐ Fatigue and mouth / bowel symptoms
- ☐ Rare serious lung or cardiac effects
- ☐ Serious infection / sepsis
- ☐ Rare serious bowel inflammation; vision changes

Discuss treatment aim, alternatives including supportive care, uncertainty, serious and lasting effects, fertility and contraception, premedication, IV access and the urgent contact plan. Explicitly document the off-label SCLC indication and weekly schedule under hospital policy; explain that the supporting study used a different weekly pattern.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00621 v3.

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